BRIGHTWATER HEALTH PLAN

Utilisation Management Policy

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1. Purpose

This policy says how Brightwater Health Plan reviews a prior authorisation request: who opens a review and who decides it, how a request is screened against its medical policy, what each decision means, which letters follow, how quickly a review is completed, and what Provider Services may tell a requesting office.

It applies to every request for the five services that need prior authorisation: MRI of the lumbar spine, MRI of the cervical spine, total knee replacement, bariatric surgery and the in-laboratory sleep study. Each is reviewed under its own medical policy.

2. Roles

- **Intake Coordinator** - opens the review of a request from the review queue: the questionnaire is read and screened, the documents are read for evidence, and the review is passed on with a recommended decision.
- **UM Nurse Reviewer** - reads a review they did not open and decides it: approves a request whose criteria are all met, asks for more information when a criterion is not documented and none is not met, or refers the request to a Physician Reviewer. A UM Nurse Reviewer never denies a request.
- **Physician Reviewer** - decides every referred request, and is the only reviewer who may deny one.
- **Provider Services Representative** - answers questions about the medical policies, this policy, the Prior Authorisation Rules and the evidence guides, and about a request only as section 10 allows.

Requesting offices and members do not decide. A requesting office may discuss a referred request with the Physician Reviewer, and receives the letters written after a decision; the member receives the notice.

3. Separation of opening and deciding

The Intake Coordinator who opens a review may not decide it. Every review is decided by a UM Nurse Reviewer who did not open it - by approving the request, asking for more information or referring it to a Physician Reviewer - and the decision is recorded with the reviewer's name and the reason, in their own words.

A recommendation, however it was produced, is only a recommendation. No letter is sent, no notice is written and no status changes until the decision is recorded. Tools may screen the questionnaire, read the documents and draft the letters, but deciding a request is never a machine's to do.

4. What is reviewed

Every request is reviewed in five parts:

1 **The questionnaire.** Each criterion of the service's own medical policy, Met, Not met or Not documented on the latest submission, read as the Prior Authorisation Rules set out.

2 **The evidence.** Every sentence of the clinical documents sent with the request, read with the evidence guide for the kind of evidence it gives. A document that disagrees with the questionnaire is flagged for the reviewer.

3 **The approval likelihood.** How often requests like this one have been approved, as section 8 describes.

4 **The expected stay.** For an inpatient operation, how many days members like this one stay in hospital, as section 8 describes.

5 **The decisions open.** The decisions the screening allows, as section 5 sets out.

Each service's criteria are read from its own medical policy and from no other document. The five medical policies share one layout and many of the same sentences, and the lumbar and cervical spine policies state their criteria in the same words, apart from the length of conservative treatment they ask for.

5. The three decisions

The UM Nurse Reviewer chooses one of three decisions, each open only as set out here:

- **APPROVE** - the request is authorised as requested, and the letter and the notice of section 6 are written. Open only when every criterion is Met.

- **REQUEST MORE INFORMATION** - the requesting office is asked for the answers the questionnaire left out. Open only when no criterion is Not met and at least one is Not documented.
- **REFER TO PHYSICIAN** - the request goes to a Physician Reviewer with its review, who decides it. Always open.

A decision the screening does not allow is refused: APPROVE while any criterion is not Met, and REQUEST MORE INFORMATION while a criterion is Not met or when nothing is missing.

The request's record moves from awaiting review to approved, information requested or referred to physician review, and keeps the review, the decision, the reviewer, the reason, the letters and the notice.

A Physician Reviewer's decision, with its reason and how to appeal it, is written to the requesting office and the member by the physician review team, outside this policy; the review's own record keeps the status referred to physician review.

6. Letters and notices

When a UM Nurse Reviewer approves a request, a provider authorisation letter is written to the requesting office and then a member notice to the member. The letter shows the request reference, the member number, the service and its code, the site of service, the medical policy the request was reviewed under and, for an inpatient operation, the inpatient days authorised. The notice tells the member, in plain words, that the service is authorised and where it is given.

When more information is requested, an information request letter is written to the requesting office. It names each criterion that was not documented and the questionnaire answers it needs, and gives the "Documentation to send" section of the service's medical policy. The office has 14 calendar days to reply, or one day for an urgent request. A request whose answers do not arrive in that time is referred to a Physician Reviewer, who decides it on what was received. No member notice is written.

A referred request has no letter and no notice until the Physician Reviewer decides. The review is recorded as referred, with a referral summary for the Physician Reviewer: the criteria, the evidence and every disagreement flagged.

No letter or notice ever contains the approval likelihood, the expected stay, another member's information or the name of a reviewer, and the notice never contains clinical detail beyond the name of the service.

7. Review timeframes and the queue

An urgent request is decided within 72 hours of arriving, and a standard request within 14 calendar days. When more information is requested, the time starts again when the office's reply arrives, or when its time to reply runs out.

A request is urgent when the requesting office says that waiting the standard time could seriously harm the member's health, as with a red flag symptom. The office marks the request urgent; a reviewer may also treat it as urgent.

The review queue puts urgent requests first. Within each urgency, requests are taken in the order of their request reference.

8. The approval likelihood and the expected stay

The approval likelihood is the share of comparable past requests that were approved, estimated by a model trained on the authorisation history. It is shown to four decimal places and banded on the figure shown: HIGH from 0.80, MEDIUM from 0.50 and LOW below that.

The expected stay is the number of days members like this one stay in hospital after the same operation, estimated by a model trained on the plan's inpatient stays. It is shown to one decimal place for an inpatient operation. When it is more than one day longer than the inpatient days requested, the review notes that a longer stay is expected, so that a review during the stay can be planned.

Neither figure decides anything: every criterion comes from the medical policy and every decision from a reviewer. Neither is ever told to a requesting office, a member or anyone outside utilisation management.

9. Privacy and clinical documents

Clinical documents are read only for utilisation review. A request's answers, documents and decision are never shared with another requesting office or another member.

Letters go only to the requesting office of the request, and the notice only to the member. The Past Reviews Casebook keeps past reviews, with members' names, as confidential teaching material for reviewers.

10. Talking to requesting offices

A Provider Services Representative confirms a request only to a caller who gives the request reference and the member number on that request. The caller is then told, for that request only, its service and its status: awaiting review, approved, information requested with the answers asked for, or referred to physician review.

Asked in any other way - with no member number, with a member number that is not the request's, or about a reference that does not exist - the representative gives the same line: "No request can be confirmed for reference" followed by the reference and a full stop, as in "No request can be confirmed for reference PA-123456." The reference is written canonically where it can be read, as the Prior Authorisation Rules set out, and as it was asked otherwise. The line never confirms that a request exists.

Asked whether a request will be approved, how likely its approval is, or how one request compares with another, the representative gives one line and nothing more: "A prior authorisation outcome is decided by clinical determination, and Provider Services cannot predict or estimate it." This line answers every such question, whether or not the caller has given the request reference and the member number, and comes before any other.

Questions about the medical policies, the Utilisation Management Policy, the Prior Authorisation Rules and the evidence guides are answered for anyone.

Intake Coordinators, UM Nurse Reviewers and Physician Reviewers may see every review, with its screening, its evidence and its decision.